A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Lauren Maisey
Northside Clinic

Dear Dr Maisey,

Re: Dr Anna Debinski
DOB: 1992-03-13
Address: 34 Delbridge Street, North Fitzroy, Vic, 3068

Summary

- **No evidence of active Crohn's disease on recent investigations including stool studies, colonoscopy, and intestinal ultrasound.**
- **Recent improvement in gastrointestinal symptoms with a return to clinical baseline.**
- **Decision to defer the planned transition to nortriptyline while the patient remains stable.**
- **No action required by you.**

I had the pleasure of conducting a review with Anna Debinski following her recent investigations. I am pleased to report that the results of her stool tests, colonoscopy, and intestinal ultrasound have collectively excluded active Crohn's disease. While she may have had active disease in the past, her current symptomatic presentation is not attributable to active inflammation. The intestinal ultrasound was particularly thorough and provides further reassurance regarding the absence of transmural inflammation.

Anna reports that her symptoms have settled significantly over the last fortnight and have largely returned to her baseline. We discussed potential exacerbating factors, including her recent family travel to Indonesia and the role of psychological stress, which she had previously identified as a possible trigger.

Impression and Plan

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Given the marked improvement in her clinical trajectory, we have elected not to precipitate any deterioration by changing her medications at this time. Although we had previously planned to transition her from amitriptyline to nortriptyline to better manage the severity and frequency of her pain episodes, we will reserve this strategy for the future should her symptoms deteriorate.

If a change to nortriptyline becomes necessary, the plan will be to titrate to a higher dose and review her clinical response six to eight weeks later. I have also determined that a CT scan to evaluate for extra-luminal pathology is not required at this stage, given her symptomatic resolution. I have asked Anna to contact me if her condition changes, but otherwise, no further immediate follow-up is scheduled

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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